

Visual Tourettic OCD: Evidence-Based Self-Help and Medications

TL;DR

- **The most effective treatment for visual tourettic OCD (VTOCD) is a combined approach — Exposure and Response Prevention (ERP) blended with Habit Reversal Training/Comprehensive Behavioral Intervention for Tics (CBIT), often augmented by a high-dose SSRI plus either a *low-dose* dopamine blocker (risperidone 0.5–2 mg/day or aripiprazole 5–15 mg/day) or an alpha-2 agonist (guanfacine 1–4 mg/day, clonidine 0.05–0.4 mg/day).** Standard OCD treatment alone is frequently inadequate — McDougle et al. (1993) found only a 21% SSRI-monotherapy response rate in OCD-with-tics vs. 52% without. ([ScienceDirect](#))
- **Practical self-help that works alongside professional care:** stop trying to suppress the eye "tic" by force; use Dr. Jonathan Grayson's "sneak-a-peek" strategy and the three-point face triangle (chin + above each eye) for natural gaze; drop reassurance-seeking, sunglasses, and avoidance; build a graded exposure ladder targeting the feared *consequence* of being caught (not the staring itself); and layer ACT-style cognitive defusion, self-compassion, and diaphragmatic breathing on top.
- **The single most important conceptual shift:** stop treating the eye urges as a problem to eliminate (which is what fuels them) and start treating them as a *tic to be tolerated* while you systematically extinguish the *OCD-driven checking, avoidance, and rumination* — the latter is what causes the suffering and impairment, not the gaze itself.

Key Findings

1. VTOCD is a recognized clinical subtype — but not in DSM-5. Visual Tourettic OCD (also called "staring OCD" or "compulsive staring") is a variant of Tourettic OCD (TOCD), a clinical framework introduced by Charles Mansueto, PhD and David Keuler, PhD in their 2005 paper *Tic or Compulsion? It's Tourettic OCD* (Behavior Modification 29(5):784–799; PMID 16046664). In VTOCD, the eyes "tic" — usually a peripheral or direct glance toward genitals, faces, scars, or other socially sensitive areas — followed by classic OCD obsessions ("What if I'm seen as a creep?") and compulsions (avoidance, mental review, reassurance-seeking, blinking hard to "reset," wearing sunglasses). The 2022 review by Katz, Bui, Worhach, Bogut, and Tomczak of Boston Children's Hospital (Frontiers in Psychiatry; doi:10.3389/fpsyt.2022.929526; PMC9363583) confirms TOCD is an "intermediate neuropsychiatric disorder distinct from either TS or OCD alone" with overlapping cortico-striato-thalamo-cortical (CSTC) circuit involvement.

(Taming Olivia + 3)

2. Standard OCD treatment is often inadequate. McDougle et al. (1993, 1994) found OCD patients with tics responded to SSRI monotherapy at only **21% vs. 52% for OCD without tics**. The NIMH-funded Pediatric OCD Treatment Study (POTS; March et al., 2007) replicated this: youth with tic-related OCD on sertraline monotherapy did *no better than placebo*, while non-tic

OCD youth responded robustly. **This is the core reason a different approach is needed for VTOCD.** (ScienceDirect) (ScienceDirect)

3. Pharmacology must address both the tic and the OCD components. Per Katz et al. 2022: *"TOCD often warrants combined treatment as well as higher dosing regimens due to overlapping symptomatology. Management with antipsychotics is required more often as compared to CTD or OCD alone."* (nih)

4. The behavioral therapy must also be modified. Mansueto and Keuler (2005) explicitly state TOCD requires *longer ERP regimens, more rote practice, deliberate "just wrong" exposure, muscle relaxation, diaphragmatic breathing, and competing-response substitution strategies* — none of which are part of standard OCD ERP protocols.

5. VTOCD has its own validated self-help techniques. Dr. Jonathan Grayson (LA Treatment Center for Anxiety and OCD; author of *Freedom from OCD*) developed the "sneak-a-peek" and "three-point gaze" techniques specifically for visual tourettic OCD, now standard recommendations from IOCDF grassroots advocates Matthew Bannister (CPSS) and Carol Edwards. (Spotify + 2)

Details

What VTOCD actually is

VTOCD presents in four overlapping patterns (per Matthew Bannister, CPSS, IOCDF grassroots advocate):

1. Repetitive involuntary glances/stares at objects in the visual periphery (Matthewbannister-cpss)
2. Fear of staring inappropriately at private body areas (without actually doing so) (Matthewbannister-cpss)
3. Believing one is staring when one is not (Matthewbannister-cpss)
4. Actually staring/glancing in ways others notice (Matthewbannister-cpss)

The *tic* component is the involuntary, urge-driven eye movement (often preceded by a "not just right" or "incompleteness" sensation, similar to a Tourette premonitory urge). The *OCD* component is the catastrophic interpretation: "What if I'm a pervert? What if I get fired/arrested/seen as creepy?" plus the compulsive responses — avoidance, mental checking, reassurance, intense face-staring to "prove" innocence, wearing sunglasses, repeated analysis of whether staring actually occurred. Per Brandt et al. (Scientific Reports, 2023; doi:10.1038/s41598-023-37658-0), "non-just-right experiences are more closely associated with comorbid OCB than with tics in patients with Tourette syndrome," which supports treating both components in parallel. (International OCD Foundat...)

Self-treatment and coping strategies

A. ERP modified for VTOCD (the core treatment)

Exposure and Response Prevention is the gold-standard treatment for OCD. Per the IOCDF page on ERP (authored by Jonathan Abramowitz, PhD): *"On average, patients achieve a reduction of 60% of OCD symptoms, along with reductions in general anxiety, depression, interpersonal stress, and functional interference."* For VTOCD specifically, the exposure target is *NOT* the staring itself — it is the feared consequence of being caught or being a "bad person." [Treat My OCD](#)

[International OCD Foundat...](#)

Practical self-applied ERP for VTOCD:

- **Build a fear hierarchy** around situations you avoid: gym, beach, public transit, meetings, life drawing classes, conversations with attractive people. Rank each 0–10 by anxiety.
- **Start at the bottom and stay** until the urge to escape drops by half (typically 20–45 minutes). The Katz 2022 review describes a TOCD-specific ladder: *"first by continuing to engage in their tics without repeating them 'just so' (i.e., tolerate doing the tics 'imperfectly'), which could then progress higher up the ladder to full suppression of tics altogether."* [nih](#)
- **Drop the rituals:** stop wearing sunglasses defensively, stop seeking reassurance ("Did I look weird?"), stop mentally reviewing whether you stared, stop avoiding eye contact altogether, stop the "blink-hard reset." [Ocdontario](#)
- **Imaginal exposure** to the feared outcome: deliberately write out "Someone saw me glance and now thinks I'm a creep" and read it daily until it loses charge.

B. Grayson's "sneak-a-peek" technique (created by Dr. Jonathan Grayson, OCD Conference July 2021 presentation, "Staring OCD or VTO")

Counterintuitively, *give yourself permission to look*. The harder you fight the urge, the stronger it gets. Grayson's specific guidance:

- At social distance (~10 ft), give yourself permission to "look — acknowledge — move on" rather than fighting the gaze [Ocdontario](#)
- At far distance, give yourself permission to stare openly (it's not a problem at distance) [Ocdontario](#)
- Do quick, deliberate "sneak peeks" rather than fighting and producing the prolonged tic-stare [Taming Olivia](#) [tamingolivia](#)
- Accept in advance: "Sometimes I stare blankly, excuse me" — and accept being caught as a possible, tolerable outcome [Ocdontario](#)

C. Three-point gaze training

To replace intense face-staring (a compulsion) with a natural look: focus on an imaginary triangle of three dots on the other person's face — chin + a point above each eye. Rotate your gaze among these slowly. This produces natural interest-display without locked staring.

[tamingolivia](#) [Taming Olivia](#)

D. Habit Reversal Training (HRT) / CBIT self-application

CBIT is recommended as first-line treatment for tics in the 2019 American Academy of Neurology Practice Guideline (Pringsheim et al., Neurology, May 7, 2019). It was validated by Piacentini et al. (JAMA, 2010;303:1929–1937) in a randomized controlled trial. Wilhelm S, Peterson AL, Piacentini J et al. (Archives of General Psychiatry 2012;69:795–803) reported: "At 6 months, 12 of the 15 (80%) available subjects in the CBIT group showed continued benefit and 1 of 4 (25%) of those in the control group showed continued benefit" — note these are percentages among participants available for follow-up, not all original enrollees. (Tourette Association of Am...)

Self-applied steps:

1. **Awareness training:** log every visual tic for one week — time, location, what preceded it (premonitory urge in eyes/forehead).
2. **Competing response training:** when you feel the urge, perform an *incompatible* behavior for one minute — e.g., deliberate slow blinking, looking at a fixed neutral point, soft-focused gaze straight ahead. The behavior must physically prevent the tic without itself becoming a ritual. (clinicaltrials) (Northwestern Medicine)
3. **Function-based intervention:** identify environments that worsen tics (small rooms, transit) and modify (sit facing a wall, take a longer-distance seat, take breaks).

E. Acceptance and Commitment Therapy (ACT) techniques

Validated for OCD by Twohig, Whittal et al. (2010, 2015). Key self-applied tools:

- **Cognitive defusion:** instead of "I'm a creep for staring," say "I'm having the thought that I'm a creep for staring." Add "...and that's just OCD doing its thing." Sing the thought to a silly tune to weaken its grip. (Counseling Center Group)
- **Acceptance** of the urge as a transient body event, not a moral signal.
- **Values-based action:** keep going to social settings, work, dating *because* they matter to you, not when anxiety has cleared. (OCD Center of L.A.)
- **Self-as-context:** you are the awareness in which OCD/tics arise — not the thoughts themselves. (Counseling Center Group)

F. Mindfulness and self-compassion

Both Catherine Benfield (Taming Olivia) and Matthew Bannister emphasize self-compassion as transformative. Kimberley Quinlan's *The Self-Compassion Workbook for OCD* (New Harbinger) is widely recommended. Practical: when the tic happens, place a hand on your chest and say "This is hard. This is OCD attacking what I value most. I'm doing my best." Studies show that ACT-derived increases in psychological flexibility and mindfulness predict OCD symptom reduction independently of CBT (Twohig et al., 2010; 2015). (Kimberleyquinlan-lmft + 2)

G. Mansueto's specific TOCD self-help additions (from the 2005 paper)

- **Diaphragmatic breathing** during exposures: imagine "breathing away" tension (behaviortherapycenter)

- **Differential muscle relaxation:** target the specific muscle group producing the urge (often eyes, brow, jaw) (behaviorthrapycenter)
- **"Just wrong" practice:** deliberately perform tics imperfectly — look slightly off-target, end the glance early, do the "wrong" number of repetitions — to build tolerance for the unresolved premonitory urge
- **Substitution strategies:** a competing motor response such as gently squeezing a fist or focusing on a specific tactile sensation when the urge arises (PubMed Central) (behaviorthrapycenter)

Mansueto and Keuler (2005) explicitly state: *"Some patients require substantial rote practice engaging in the 'just wrong' versus the 'just right' behavior before they experience any reduction in tension. As such, the clinician and the patient must be prepared for a longer regimen of exposure and response prevention."* (behaviorthrapycenter)

H. What NOT to do

- Don't wear sunglasses defensively (compulsion) (Ocdontario)
- Don't avoid social situations (compulsion) (Ocdontario)
- Don't seek reassurance from friends/partners ("Was that weird?") (Ocdontario)
- Don't compulsively google or post about VTOCD seeking certainty
- Don't try to suppress every urge — this often makes them stronger (paradoxical rebound)
- Don't take cannabis as a self-treatment; though the AAN guideline notes some adults with TS use it, it should only be physician-supervised in legal states and can worsen OCD anxiety in many users (AAN) (AAN)

Medications

Note: All medication decisions must be made with a psychiatrist. Doses below are typical adult ranges for the conditions described.

First-line: SSRI monotherapy at OCD-level doses (12-16 week trial)

Per the International OCD Foundation, the FDA-approved SSRIs for OCD are fluoxetine, sertraline, paroxetine, fluvoxamine, plus clomipramine (a TCA). Citalopram and escitalopram are off-label but effective. SSRIs for OCD require *higher doses* than for depression, and a *longer trial* (8-12 weeks minimum, with 6 weeks at therapeutic dose). (nih + 3)

Typical OCD doses (per IOCDF and APA practice guidelines):

- **Fluoxetine (Prozac):** 40-80 mg/day (APA permits occasional dosing up to 120 mg/day) (Psychiatrist) (PubMed Central)
- **Sertraline (Zoloft):** 200-400 mg/day (FDA max for depression is 200 mg; OCD often requires up to 400) (Psychiatrist) (Psychopharmacology Instit...)
- **Fluvoxamine (Luvox):** 200-300 mg/day (APA permits up to 450 mg/day) (Psychiatrist) (PubMed Central)

- **Paroxetine (Paxil):** 40–60 mg/day (APA permits up to 100 mg/day; less preferred due to side-effect burden) (Psychiatrist + 2)
- **Escitalopram (Lexapro):** 10–20 mg/day (APA permits up to 60 mg/day; QT-prolongation concerns above 20) (Psychiatrist + 3)
- **Clomipramine (Anafranil):** 100–250 mg/day (often slightly more effective than SSRIs but heavier side effects) (Psychopharmacology Instit...)

Important caveat for VTOCD: McDougle et al. (1993) showed a 21% SSRI-monotherapy response in OCD-with-tics vs. 52% without. Expect partial response and plan for augmentation early. (ScienceDirect)

Second-line: SSRI + low-dose antipsychotic augmentation

This is the most evidence-based augmentation strategy. Per Pittenger C and Bloch MH (Psychiatric Clinics of North America 2014;37:375–391; PMC4143776): *"Approximately one-third of treatment refractory OCD patients will respond to antipsychotic augmentation; the NNT is 4.6. OCD patients with comorbid tic disorders appear to respond particularly well to antipsychotic augmentation."* The foundational meta-analysis is Bloch MH, Landeros-Weisenberger A, Kelmendi B, Coric V, Bracken MB, Leckman JF, "A systematic review: antipsychotic augmentation with treatment refractory obsessive-compulsive disorder" (Molecular Psychiatry 2006;11:622–632; doi:10.1038/sj.mp.4001823). McDougle's pioneering trial showed that fluvoxamine + haloperidol outperformed fluvoxamine alone *specifically in patients with comorbid tics*. (PubMed Central + 2)

The UK Advanced Interventions Service explicitly warns against high-dose augmentation: doses of risperidone 6–8 mg/day caused side effects (drug-induced Parkinsonism, marked sedation) without added benefit. Effective ranges from the literature: (Advancedinterventions)

- **Risperidone (Risperdal):** 0.5–3 mg/day for OCD augmentation; ~2.2 mg/day was effective in Yale's Scahill RCT for tics, with 32% YGTSS reduction vs. 7% placebo (nih + 2)
- **Aripiprazole (Abilify):** 5–15 mg/day for OCD augmentation; AAN tic guidelines suggest 5 mg/day if <50 kg or 10 mg/day if ≥50 kg; mean 4.5 mg/day in pediatric Tourette open-label trials (nih) (PubMed Central)
- **Haloperidol (Haldol):** low-dose 1–3 mg/day (mean 6.2 mg in McDougle's original study); FDA-approved for tics but extrapyramidal side-effect risk (Psychiatric Times) (PubMed Central)
- **Pimozide (Orap):** FDA-approved for tics but ECG monitoring required (QT prolongation)
- **Quetiapine (Seroquel):** mean ~169 mg/day in OCD augmentation studies (less evidence than risperidone/aripiprazole) (Psychiatric Times)
- **Olanzapine (Zyprexa):** mean ~6 mg/day (less preferred due to metabolic side effects) (Psychiatric Times)
- **Brexipiprazole (Rexulti):** emerging evidence at 1–4 mg/day (Cuomo et al. 2024 retrospective; PMC12459139); not yet a guideline-level recommendation

Alpha-2 agonists (first-line for tic component)

Per the 2019 AAN guidelines (Pringsheim, Okun, Holler-Managan et al., Neurology May 7, 2019) and European clinical guidelines for Tourette syndrome v2.0 (Roessner et al., Eur Child Adolesc Psychiatry 2022;31:425-441), alpha-2 agonists are first-line for the tic component because they have lower side-effect burden than antipsychotics. They are particularly well-suited to TOCD/VTOCD because Mansueto and Keuler specifically recommend them — yet they are often overlooked when no visible motor tic is present. (Neurodivergencewales + 4)

- **Guanfacine (Tenex/Intuniv):** start 0.5 mg at bedtime, titrate by 0.5 mg/week to 1.5-4 mg/day divided twice daily. Less sedating than clonidine. Per Mansueto's case series, a 10-year-old TOCD patient who failed fluvoxamine 150 mg responded to guanfacine at standard doses. (PubMed + 3)
- **Clonidine (Catapres):** start 0.05 mg at bedtime, titrate by 0.05 mg every 4-7 days to 0.1-0.4 mg/day in 2-4 divided doses. Comes as oral or transdermal patch. Common side effects: sedation, dry mouth, hypotension. Do not stop abruptly (rebound hypertension). (Amegroups) (Great Ormond Street Hosp...)

Treatment-resistant TOCD

Per the Katz et al. 2022 algorithm — if SSRI + antipsychotic augmentation + behavioral therapy fails after 3 months at therapeutic doses (defined as "less than a 25-35% reduction on the Child Yale Brown Obsessive Compulsive Scale"): (PubMed Central) (nih)

- Switch SSRIs (try 2-3 before declaring true treatment-resistance)
- Try clomipramine (if not already)
- Try supratherapeutic-dose SSRI under close supervision (per Gaviria et al. narrative review 2024, PMC12155805)
- Consider deep brain stimulation (DBS) — for adults only, targets include nucleus accumbens/anterior limb of internal capsule for OCD or CM-Pf thalamus for tics; the Katz review cites a 65-study meta-analysis where ~70% of TS patients achieved >50% YGTSS reduction (nih)
- Off-label considerations: topiramate — Jankovic J, Jimenez-Shahed J, Brown LW (J Neurol Neurosurg Psychiatry 2010) reported in a randomized, double-blind, placebo-controlled parallel-group study in 29 Tourette patients (mean age 16.5 years): "the YGTSS total tic score improved by 14.29 points from baseline to visit 5 (day 70) with topiramate (mean dose of 118 mg) compared with a 5.00 point change in the placebo group (p = 0.0259)" (Sage Journals) (Aetna)
- Active research: the NYU Langone trial of ondansetron for sensory phenomena in OCD/tics (ClinicalTrials.gov NCT03239210) is exploring this overlap pharmacologically

How VTOCD/TOCD treatment differs from standard OCD treatment

Dimension	Standard OCD	Visual Tourettic OCD
First-line therapy	ERP alone	ERP + CBIT/HRT in parallel

Dimension	Standard OCD	Visual Tourettic OCD
ERP modifications	Standard hierarchy	Longer sessions, more rote practice, deliberate "just wrong" practice, relaxation/breathing/imagery, competing responses
SSRI monotherapy response	~52% (McDougale 1993)	~21% (McDougale 1993)
First-line meds	SSRI alone	Often SSRI + low-dose alpha-2 agonist or antipsychotic considered earlier
Augmentation timing	After SSRI fails	May start combined therapy earlier given known poor SSRI response
Role of alpha-2 agonists	Not used	First-line for the tic component (clonidine, guanfacine)
Polypharmacy	Less common	Per Katz 2022: <i>"more likely to require polypharmacy and higher dosing regimens"</i> (nih)
Exposure target	The obsessional fear	The fear + tolerating the unresolved "not just right" sensation

Recent research and clinical guidelines

- **Katz, Bui, Worhach, Bogut, Tomczak (2022)** — *Frontiers in Psychiatry, "Tourettic OCD: Current understanding and treatment challenges of a unique endophenotype"* — the most current authoritative review; provides the consensus treatment algorithm
- **American Academy of Neurology Practice Guideline (May 2019)** — Pringsheim et al., *Neurology*: CBIT is first-line for tics in TS and chronic tic disorders, ahead of medication (Sage Journals +2) (nih)
- **European clinical guidelines for Tourette syndrome v2.0** (Roessner et al., 2022) — *Eur Child Adolesc Psychiatry* — pharmacological recommendations align with AAN
- **IOCDF blog "Tics, OCD, and Tourettic OCD"** (Christine Conelea, PhD, University of Minnesota MnTiC Lab, May 2025) — confirms *"people with Tourettic OCD may not respond as well to our first-line treatments, like medication for OCD and Cognitive Behavioral Intervention for Tics (CBIT)"* and that research is moving toward circuit-specific treatments (International OCD Foundat...)
- **IOCDF Visual Tourettic OCD Warrior Badass Tribe** — virtual peer support group founded by Matthew Bannister, CPSS (IOCDF grassroots advocate) (International OCD Foundat...)
- **Brandt et al. (2023)** in *Scientific Reports* found that "not-just-right" experiences correlate more closely with OCD than with tics in Tourette patients — supporting combined treatment (nih)

Staged approach for someone with VTOCD:

Stage 1 (Self-help baseline, do this now while seeking a specialist):

1. Read Mansueto & Keuler's 2005 paper (free on the Behavior Therapy Center of Greater Washington website) and the Katz 2022 review (open access at PMC9363583).
2. Start a daily 10-minute mindfulness or self-compassion practice (Kimberley Quinlan's *Self-Compassion Workbook for OCD* is the best fit).
3. Stop the obvious compulsions: sunglasses, avoidance, reassurance-seeking, mental review, blink-resetting.
4. Begin Grayson's three-point face triangle and sneak-a-peek in low-stakes situations.
5. Build a written fear hierarchy and start exposures at level 2-3.
6. Buy and work through Carol Edwards' *Address Staring OCD: How To Manage Visual Tics And Obsessions* (Edwards is a CBT-trained writer with lived VTOCD experience). [Amazon](#)
7. Join the IOCDF Visual Tourettic OCD Warrior Badass Tribe support group.

Stage 2 (Within 4-8 weeks — find a specialist): 8. Find a clinician trained in *both* ERP for OCD and CBIT for tics. The IOCDF provider directory and the Tourette Association of America's Behavior Therapy Institute trainee list are the two best sources. 9. Ask the clinician explicitly: "Are you familiar with Tourettic OCD as described by Mansueto?" If not, find another clinician. 10. Begin combined ERP + HRT with weekly sessions; expect a longer course (16-20+ weeks) than typical OCD ERP.

Stage 3 (If symptoms remain moderate-severe after 8-12 weeks of therapy): 11. Add an SSRI — sertraline or fluoxetine, titrated to OCD doses (200-400 mg/day or 60-80 mg/day respectively) over 12 weeks. 12. If only partial response and prominent tic features, add either guanfacine (1-3 mg/day) or low-dose aripiprazole (2-10 mg/day) or risperidone (0.5-2 mg/day). Discuss the alpha-2 agonist option first if side-effect tolerance is the priority.

Stage 4 (Treatment-resistant — <25-35% Y-BOCS reduction after 3 months at therapeutic doses): 13. Switch SSRIs (try at least 2 different SSRIs); consider clomipramine. 14. Consider suprathreshold SSRI dosing under close supervision. 15. Consider referral to a specialist OCD program with access to TMS or — for severe adult cases — DBS evaluation at a tertiary center.

Benchmarks that should change the plan:

- *No improvement at 12 weeks of well-implemented ERP+CBIT* → add or change medication.
- *<25% Y-BOCS reduction after 3 months on therapeutic-dose SSRI* → add an antipsychotic or alpha-2 agonist (per Katz 2022 and APA guidelines).
- *Functional impairment persists despite SSRI + augmentation + therapy at 6-9 months* → refer to a tertiary OCD/Tourette specialist center.
- *Suicidal ideation, complete social withdrawal, or job loss* → urgent psychiatric referral; do not stay in stage 1 self-help.

Caveats

- **VTOCD is not in the DSM-5.** It remains a clinical, not a diagnostic, label. Some clinicians may not recognize the term; using "tic-related OCD" or "OCD with a comorbid eye/staring tic" may communicate better. (PubMed Central) (nih)
- **No randomized controlled trial has been done on VTOCD specifically.** All treatment recommendations are extrapolated from tic-related OCD studies (McDougle, POTS, Maina) and from clinical experience (Mansueto, Grayson, Katz/Tomczak). The 2014 Conelea study estimated ~50% of OCD patients have had a tic at some point, but how many have the specific VTOCD presentation is unknown.
- **Pediatric vs. adult treatment differs.** Most TOCD evidence is pediatric (Katz et al. is a pediatric review). Adult VTOCD is under-studied; dosing should be conservative.
- **Antipsychotic side effects are real.** Risperidone causes weight gain and prolactin elevation; aripiprazole can cause akathisia; haloperidol carries extrapyramidal and tardive dyskinesia risk. The UK Advanced Interventions Service explicitly warns against the high-dose augmentation regimens (risperidone 6–8 mg/day) that some clinicians use. Stay low. (Psychiatric Times)
- **SSRIs in young people require monitoring** for treatment-emergent suicidality (FDA black box warning under age 25).
- **Self-help is not a substitute for treatment** when impairment is severe. If you are losing your job, isolating, or having suicidal thoughts, seek immediate professional help.
- **Cannabis** is mentioned in the AAN guideline as something some adults with TS use for tics, but with caution and only under medical supervision in legal states. It can *worsen* OCD obsessions and anxiety in many users. (AAN)
- **Online sources vary in quality.** SEO-driven sites contain accurate-sounding but unsourced material. Rely on IOCDF.org, Tourette.org, AAN guidelines, PubMed/PMC, and named clinicians (Mansueto, Grayson, Bannister, Edwards, Conelea, Quinlan, Katz/Tomczak).
- **Stimulants for comorbid ADHD** — most don't worsen tics meaningfully, but high-dose dextroamphetamine has been shown to exacerbate them (per Katz 2022). Methylphenidate, guanfacine, clonidine, atomoxetine all have evidence for tic-ADHD comorbidity. (nih) (nih)
- **The Wilhelm/Piacentini CBIT outcome figures** are derived from small follow-up subsamples (15 CBIT, 4 control); the impressive 80% durability rate has not been replicated in larger long-term studies and should be interpreted as preliminary rather than definitive.